

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A RANDOMIZED, 2-ARM, PHASE 3 STUDY OF ELRANATAMAB (PF-06863135) VERSUS LENALIDOMIDE IN PATIENTS WITH NEWLY DIAGNOSED MULTIPLE MYELOMA AFTER UNDERGOING AUTOLOGOUS STEM-CELL TRANSPLANTATION
Short Title:	Study With Elranatamab Versus Lenalidomide in Patients With Newly Diagnosed Multiple Myeloma After Transplant
Protocol Number:	1027326502524431723
NCT Number:	NCT05317416
Sponsor Name:	Pfizer
Investigational Product:	lenalidomide
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Minimal Residual Disease negativity rate: Minimal Residual Disease negativity rate per IMWG criteria as assessed via Next Generation Sequencing 2. Progression Free Survival: Progression Free Survival assessed by Blinded Independent Central review per IMWG response criteria
Secondary Obj.:	1. Overall Survival 2. Sustained MRD negative rate 3. Progression Free Survival

2.2 Background & Rationale

The purpose of this study is to evaluate whether elranatamab monotherapy can provide clinical benefit compared to lenalidomide monotherapy (control) in participants with newly diagnosed multiple myeloma after undergoing autologous stem cell transplant. In Part 1 and Part 2 of the study, participants in the study will either receive elranatamab (arm A and C) as an injection under the skin at the study clinic or lenalidomide orally once daily at home (arm B). Participation in the study will be approximately five years

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	760

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Diagnosis of MM as defined according to IMWG criteria (Rajkumar, 2014) with measurable disease at diagnosis
- * Part 1 patients must be MRD positive, Part 2 patients can be MRD negative or MRD positive
- * History of induction therapy for newly diagnosed MM, followed by high dose therapy and autologous stem cell

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transplant. Randomization must occur within 120 days from the stem cell transplant. For participants who receive consolidation therapy after ASCT, randomization must occur within 60 days of consolidation and within 7 months from ASCT.

- * Partial Response or better according to IMWG criteria at the time of randomization
- * Must have an archival bone marrow aspirate sample(s) to identify the dominant malignant (index) clone by central laboratory NGS test (ClonoSEQ assay) that is used to track MRD status. This sample should preferably be collected before induction treatment (eg, at diagnosis) or before transplant.
- * ECOG performance status ?1
- * Resolved acute effects of any prior therapy to baseline severity or CTCAE Grade ? 1
- * Not pregnant and willing to use contraception

4.2 Exclusion Criteria

- * Plasma cell leukemia
- * Amyloidosis, Waldenström's macroglobulinemia
- * POEMS syndrome
- * Known active CNS involvement or clinical signs of myelomatous meningeal involvement
- * Previous MM maintenance treatment
- * Prior treatment with BCMA targeted therapy
- * Any other active malignancy within 3 years prior to enrollment, except for adequately treated basal cell or squamous cell skin cancer, or carcinoma in situ
- * Active, uncontrolled bacterial, fungal, or viral infection, including (but not limited to) HBV, HCV, and known HIV or AIDS-related illness
- * Previous administration with an investigational drug or vaccine within 30 days (or as determined by the local requirement) or 5 half-lives preceding the first dose of study intervention used in this study (whichever is longer)

11. Study Identifiers & Governance

Primary ID:	1027326502524431723
Registry Number:	NCT05317416
Sponsor:	Pfizer
Study Title:	Study With Elranatamab Versus Lenalidomide in Patients With Newly Diagnosed Multiple Myeloma After Transplant

15. Sign-off & Approval

Principal Investigator: _____	Date: _____
Clinical Operations Lead: _____	Date: _____
Lead Statistician: _____	Date: _____